MINISTRY OF JUSTICE · ARUBA

One health and support service for every justice institution

How the centralized medical department works in practice for KIA, KPA and Guarda Nos Costa

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THE MINISTER'S DIRECTION

From a police health plan to a Ministry-wide service

STRATEGIC PLAN, JUNE 2026

- Medical care for the 4 KPA stations and Guarda Nos Costa
- An extension of the DGWA medical department at KIA
- 2 extra physicians and 2 extra nurses
- On site 08:00–16:00, on call 24/7

DIRECTION FROM THE MINISTER

- One medical department, centralized under the Ministry of Justice
- Serves every justice institution that needs it
- Family Justice becomes the umbrella, under a new name
- Adds Slachtofferhulp, other professionals and a community case commission

FOR THE MINISTER TO CHOOSE

Twelve names, easy to say, about helping the community

Recht op Zorg

NL

“Right to care”: recht means both justice and right.

Hart voor Recht

NL

“Heart for justice”: care with a human face.

Justitie Dichtbij

NL

“Justice close by”: help that comes to the community.

Steunpunt Justitie

NL

“Justice support point”: one place to turn to.

Huis van Zorg en Recht

NL

“House of care and justice”: one roof for all services.

De Brug

NL

“The bridge”, also in Papiamento: justice, care and community.

Dienst Zorg & Welzijn Justitie

NL

Formal government style (DZWJ).

Hustisia Cuida

PAP

Papiamento: “Justice cares”.

Man na Man

PAP

Papiamento: “hand in hand”, justice and community together.

CORA

ES

Centro de Orientación, Rehabilitación y Atención; echoes “corazón”.

JUSTA

EN · ES

Justice Unified Support, Treatment & Assistance; “fair” in Spanish.

Justice Care Aruba

EN

Clear for international partners (JCA).

Until a name is chosen, this presentation calls it “the Central Service”.

STRUCTURE

Four services under one roof

Family Justice becomes the umbrella, under its new name

Central Medical Department

Physicians, nurses, psychiatry, psychology, addiction care and orthopedagogy for every justice institution

Slachtofferhulp

Victim support, working side by side with the medical and psychological team

Other professionals

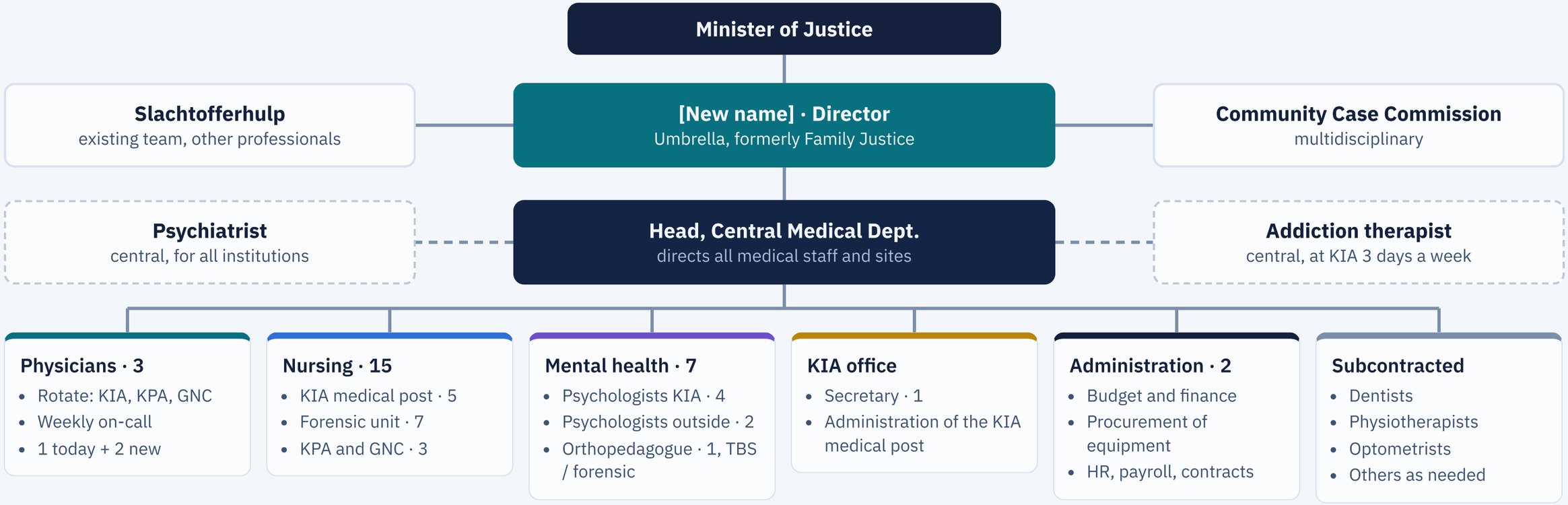
Family Justice staff and further disciplines [to define]

Community Case Commission

Analyzes problematic community cases and gives each one an individual route to the right care

ORGANIZATION CHART

Central Medical Department



WHO WE SERVE

Four doors into one service

KIA / DGWA

Prison

- Medical post
- Forensic psychiatric unit
- Psychology, addiction care, orthopedagogy

KPA

Police

- Playa, Santa Cruz, Noord, San Nicolas
- Lands Recherche and Wayaca on request

GNC

Guarda Nos Costa

- Migrant intake
- Disease screening
- Pre-deportation clearance

NEW

Other justice institutions

- On request, when the case requires it
- Community cases via the commission

CONTACT PERSONS

A contact person and a deputy in every institution

KIA / DGWA

Contact person + deputy

Medical post, forensic unit, access and security

KPA

Contact person + deputy

Stations, cars, fuel vouchers, detainee transport

GNC

Contact person + deputy

Intake, interpreters, deportation planning

OTHER INSTITUTIONS

Contact person + deputy

Named when the institution joins the service

- One person to call when a process is not flowing
- Solves problems with access, transport, equipment, medication and vouchers
- Meets the Head of the Medical Department every month
- The deputy always covers leave and absence

- The Central Service has its own: the Head of the Medical Department, with a deputy. Names and phone numbers go into each MoU and the administration keeps them up to date.

STAFFING

From 9 people today to 31, plus the Slachtofferhulp team

Role	Today	Target	New	Where they work
Head, Central Medical Dept.	1	1	–	Central, all sites
Psychiatrist	1*	1	–	Central, all sites
Secretary	1	1	–	KIA office
Attending physicians	1	3	2	Rotating: KIA, KPA, GNC
Nurses, KIA medical post	3	5	2	KIA, fixed
Nurses, forensic unit	0	7	7	KIA forensic unit, day and night shifts
Nurses, KPA and GNC	0	3	3	2 KPA circuits, 1 GNC
Psychologists, KIA	2	4	2	KIA, fixed
Psychologists, external	0	2	2	KPA, GNC, other institutions
Addiction therapist	0	1	1	Central; KIA 3 days a week
Orthopedagogue	0	1	1	TBS, or KIA forensic unit
Administration	0	2	2	Central office: budget, procurement, HR
Slachtofferhulp, existing team	existing	existing	–	Government staff already in place; no new budget
Total	**9**	**31**	**22**	*** psychiatrist being appointed**

TO START THE PROJECT

22 new positions

31 staff paid by the Central Service, plus the existing Slachtofferhulp team

12
nurses
7 forensic · 3 KPA/GNC · 2 KIA

4
psychologists
2 KIA · 2 external

2
physicians
already identified in Aruba

1
addiction therapist
central, KIA 3 days a week

1
orthopedagogue
TBS or forensic unit

2
administration
budget, purchasing, HR

WHO STAYS, WHO MOVES

Fixed teams, rotating physicians, central leadership

FIXED AT KIA

Do not rotate

- 5 nurses, medical post
- 7 nurses, forensic unit
- 4 psychologists
- Orthopedagogue, TBS or forensic unit
- Secretary, KIA office

ROTATING

All sites

- 3 attending physicians
- Rotate KIA, KPA, GNC
- Take turns on the weekly on-call

EXTERNAL BASE

Outside KIA

- 3 nurses, KPA and GNC
- 2 external psychologists
- KIA nursing backs them up, and the reverse

CENTRAL

Move to work and supervise

- Head of the department
- Psychiatrist
- Addiction therapist, KIA 3 days a week

- On call every week, each with an on-call phone: one physician, one nurse, one psychologist and the psychiatrist. Not on call: the addiction therapist and the orthopedagogue.

A NORMAL WORKING DAY

Who is where, every day

Site	Hours	Physician	Nursing	Mental health
KIA medical post	Mon–Fri 08–16, KIA roster	Physician 1	5 nurses, KIA roster	4 psychologists; addiction therapist Mon, Wed, Fri
KIA forensic unit	24 hours, 7 days: 07–15, 15–23, 23–07	Physician 2; psychiatrist by schedule	2 nurses by day, 1 in the evening, 1 at night	Orthopedagogue; KIA psychologists
KPA North: Playa, Noord	Mon–Fri 08–16	Physician 3, external circuit	1 nurse	External psychologist on request
KPA South: Santa Cruz, San Nicolas	Mon–Fri 08–16	Physician 3	1 nurse	External psychologist on request
Guarda Nos Costa	Mon–Fri 08–16	Physician 3	1 nurse	External psychologist, intake screen
After hours	**16–08, weekends, holidays**	**On-call physician of the week**	**On-call nurse of the week**	**On-call psychologist of the week; psychiatrist by phone**

Proposed: forensic shift hours, the KPA North/South split and the addiction therapist’s KIA days. Allocation follows detainee volume and acuity.

ROTATION

A three-week cycle for physicians and on-call

Position	Week 1	Week 2	Week 3
KIA medical post	Dr. A	Dr. B	Dr. C
KIA forensic unit	Dr. B	Dr. C	Dr. A
External circuit + on-call	Dr. C	Dr. A	Dr. B
On-call nurse, KPA/GNC	Nurse 1	Nurse 2	Nurse 3
On-call psychologist	Psych. 1	Psych. 2	Psych. 3
Psychiatrist, by phone	Psychiatrist	Psychiatrist	Psychiatrist

- Each physician is on call one week in three
- 4 on-call phones: physician, nurse, psychologist, psychiatrist
- Any physician covers any site during absences

Example roster. The psychologists' on-call rotation is defined with the team. KIA posts can rotate monthly for continuity.

PSYCHIATRIC DETAINEES

No TBS in Aruba: two options for proper care

OPTION A · BEST PRACTICE

A forensic care institution

- Similar to the Dutch TBS clinics, outside the prison
- Forensic psychiatrists, psychologists and nurses
- Needs its own budget, building and specialists
- Start with a feasibility study

OPTION B · ACHIEVABLE NOW

Reinforce the unit inside KIA

- 7 nurses covering 24 hours, day and night
- Day 07–15: 2 nurses · evening 15–23: 1 · night 23–07: 1
- Every nurse rotates through day and night shifts: a condition of the contract
- KIA psychologists: therapy, risk assessment and crisis support
- Daily physician, psychiatrist by schedule, orthopedagogue

- **The math:** 28 shifts a week to fill (14 day, 7 evening, 7 night); 7 nurses give 35, leaving 20% for leave, sickness and training. Two nurses on every shift later would need about 3–4 more nurses.

Assumes 5 shifts of 8 hours per nurse per week [to confirm with HR rules].

CARE FLOW

One route to care, at every site, day and night

1

Need

Detainee, staff or the intake screen flags a health need

2

Nurse

08–16: the site nurse evaluates. After hours: the duty officer calls the on-call nurse

3

Physician

On site or on call: phone advice, visit, prescription

4

Outcome

Treat on site, psychology or psychiatry, subcontracted care, HOH/IMSAN, ambulance

- Every step is recorded once in the electronic medical record and is visible at the next stop of the custody chain.

Ambulance criteria, referrals and waiting-time targets as in the June plan, sections 4.3–4.7.

COMMUNITY CASE COMMISSION

Every difficult case gets a name, a plan and an owner

Signal
From KPA, KIA, GNC, Slachtofferhulp or the community

Screening
Intake and file; urgent cases fast-tracked

Review
The commission meets on a fixed cycle

Plan
Individual plan and one case owner

Follow-up
Progress reviewed until the case is closed

PROPOSED MEMBERS

Head of the Medical Department (chair) · Psychiatrist · Psychologist · Addiction therapist · Orthopedagogue · Slachtofferhulp · Family Justice professional · Liaison of the referring institution

Meeting frequency, admission criteria and confidentiality rules go in the commission’s regulations [to define].

ELECTRONIC MEDICAL RECORDS

One record that follows the person through the justice chain

Arrest
KPA station intake

Detention
KIA or GNC see the history

Care
Prescriptions, referrals, psychology

Exit
Release, transfer or deportation

Why it matters

- Lower cost: no repeated histories, tests or avoidable transfers
- Continuity: the next institution sees what the last one did
- Indicators and IVA reports come out automatically

What the system must offer

- Built for custody: cell, status, transfers, pre-deportation clearance
- Medical data kept apart from custodial data, access by role
- 4 languages, encryption, backups, data location set by the Ministry
- Access from Microsoft Surface with MiFi for the on-call team

The choice of platform is a Ministry decision. It should be treated as a core element of the plan, like staffing.

WHO PAYS WHAT

The Central Service pays for people; institutions pay for their sites

CENTRAL SERVICE BUDGET

- All salaries, including shift and on-call allowances
- Continuing education of all staff
- Basic tools: psychological tests, stethoscopes, otoscopes, thermometers, oximeters
- 3 cars: Head, psychiatrist, on-call psychologist
- 4 on-call phones: physician, nurse, psychologist, psychiatrist
- 4 Microsoft Surface with MiFi for the on-call team
- Phone allowance (telefoontoelage) for all other staff
- Administration: 2 staff for budget, procurement, HR
- Contracting of external providers
- Electronic records [cost split to confirm]

EACH RECEIVING INSTITUTION

- Offices, building and office supplies
- Medical devices: ECG, AED, electrocautery, beds, fridges
- Referrals: hospital (HOH), IMSAN, specialists, dentists and other medical providers
- Medication: KIA pays its own; KPA and GNC, decision pending
- KPA: 2 cars for the on-call physician and nurse
- Weekly fuel vouchers

One MoU per institution: KIA, KPA, GNC and every institution that joins. Priced budget within 30 days of approval.

MEDICATION · DECISION NEEDED

Who pays for medication at KPA and Guarda Nos Costa?

THE PROBLEM

- The Botica di Servicio pharmacies froze the KPA and GNC accounts because of irregular payments
- Guarda Nos Costa has no budget for medication
- Without medication, detainees cannot be treated on site and end up in hospital

OPTION A

A budget line in each institution

KPA and GNC each receive their own medication budget and pay the pharmacy on time

OPTION B · PROPOSED

One budget in the Central Service

Medication only: the Central Service orders and pays, and its administration guarantees on-time payment to the pharmacy

OPTION C

A Ministry medication fund

One fund for the medication of every detainee, managed by the Central Service

- KIA keeps paying its own medication. Referrals to the hospital, IMSAN, specialists, dentists and other medical providers stay with each institution. In every option: settle the frozen accounts, agree fixed payment terms with the pharmacy and keep the annual review of the medication list.

Decision of the Minister. Until then, medication for KPA and GNC detainees has no secure source of payment.

ADMINISTRATION

Two administrative staff to run the Central Service

Budget and finance

- Prepares and manages the annual budget
- Salaries, shift and on-call allowances
- Reports spending to the Ministry

Procurement and equipment

- Buys the department’s own medical equipment and tests
- Keeps the inventory and the 3-year renewal cycle
- Runs phones, cars and fuel

People

- HR and payroll for all staff
- Rosters, on-call weeks and leave
- Certifications and continuing education records

Contracts

- MoUs with KIA, KPA, GNC and other institutions
- Contracts with subcontracted providers
- Invoices and cost recovery

- Clinicians spend their time on patients, not on paperwork: one office handles money, purchasing and people for every site.

Two staff: one for budget and finance, one for procurement, HR and contracts [profiles to confirm with the Ministry].

MOBILITY

Cars, phones and devices for every site

All staff

Travel to work with their own transport

1 car · KIA

Already assigned to the nurses on duty for KIA

2 cars · KPA

On-call physician and on-call nurse of the week

3 cars · Central

New: Head, psychiatrist and on-call psychologist. Owned by the Central Medical Department

- **4 on-call phones** for the physician, the nurse, the psychologist and the psychiatrist on call, handed over with each weekly rotation. All other staff use their own mobile phone and receive a phone allowance (telefoontoelage).

- **4 Microsoft Surface with MiFi** (laptop and tablet in one) for the on-call physician, nurse, psychologist and psychiatrist, to open the electronic medical record from any site.

- **Fuel vouchers every week**, delivered by the receiving institutions, so the service never stops for lack of fuel.

Fuel and maintenance of the 3 central cars from the Central Service budget [to confirm].

QUALITY

Trained staff, one contracting point

Continuing education budget

- BLS, ACLS, PHTLS and ATLS for physicians and nurses
- Forensic psychiatry training for the forensic unit
- Updates for psychology, addiction care and orthopedagogy
- Certifications tracked centrally; none may expire

Subcontracted services

- Dentists, physiotherapists, optometrists and others
- Contracted by the Central Service for all institutions
- One price list, one quality standard
- Paid by the receiving institution

IMPLEMENTATION

Twelve months, five phases

Phase	When	What happens
0 · Decision	Month 1	Ministerial decision, name chosen, umbrella designated, 2 physicians appointed, vacancies opened, contact persons and deputies named
1 · Foundation	Months 2–3	MoUs with KIA, KPA and GNC; KIA protocols adapted; records platform chosen; administration unit set up; 5 cars, 4 phones and 4 Microsoft Surface with MiFi assigned; recruitment
2 · KIA and KPA	Months 4–6	KIA reinforced (nurses, psychologists, addiction care); KPA circuits and on-call week start
3 · GNC and forensic	Months 7–9	GNC nurse in place; forensic unit staffed 24 hours, day and night, with the orthopedagogue; commission working
4 · Review	Months 10–12	First indicator report; decision on 2 nurses per shift and on a forensic care institution

Medical attention at KPA can start in Phase 0 with the 2 physicians, 2 cars and 2 on-call phones, as in the June plan.

DECISIONS REQUESTED

Seven decisions to start

1. Centralize the medical department under the Ministry, with Family Justice as the renamed umbrella
2. Choose the name of the new department
3. Approve the 22 new positions, starting with the 2 physicians who are ready
4. Give the Central Service its own budget and administration unit, with 3 cars, 4 on-call phones and 4 Microsoft Surface with MiFi
5. Instruct KIA, KPA and GNC to sign MoUs: sites, equipment, cars, weekly fuel vouchers, a contact person and a deputy
6. Reinforce the KIA forensic unit now and study a forensic care institution
7. Decide which budget pays for medication at KPA and Guarda Nos Costa

Within 30 days of approval: priced budget, draft MoUs, appointment recommendations and the Phase 1 work plan.